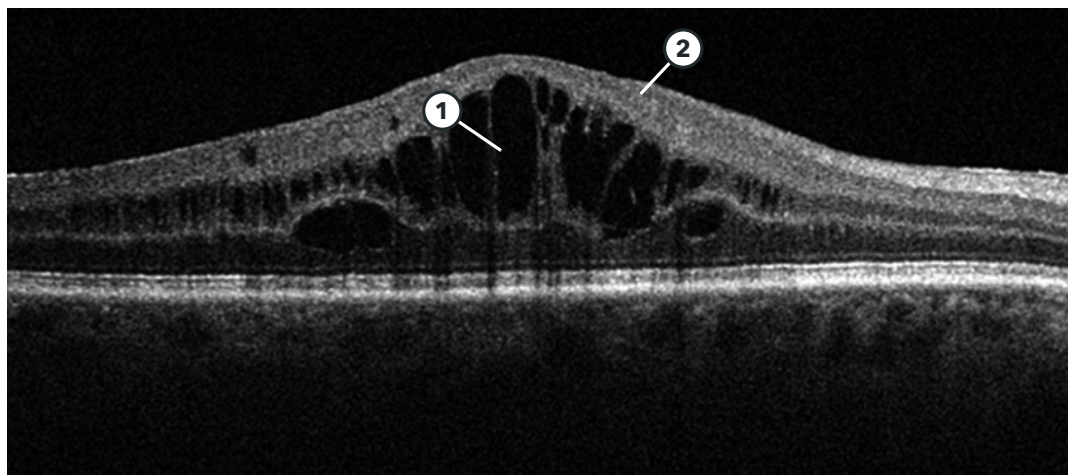


DME: centre, vision and perfusion

Clinical M3-M5 | Focused Type B notes

OCT fluid is one part of the decision

Real OCT: fluid within retinal tissue



1: intraretinal cystoid spaces. 2: thickened central retina. The scan shows anatomy; acuity and centre involvement still determine treatment. Supplied Clinical 01 Quiz Series, p 3.

Treatment branch under NICE NG 242

Pattern	Initial discussion
Noncentral clinically significant DME	Offer macular laser.
Centre-involving + good vision	Consider observation or macular laser; maintain review.
Centre-involving + impaired vision	Discuss anti-VEGF; apply local eligibility and retinal-thickness criteria.

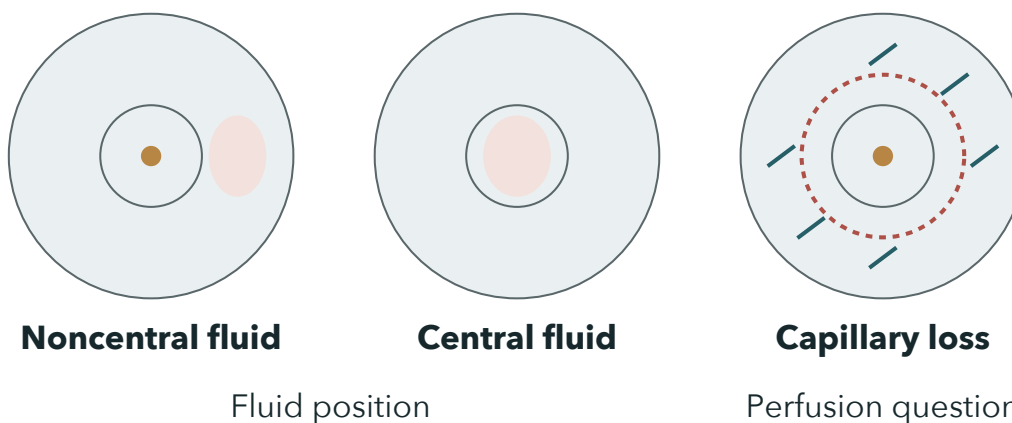
Keep two labels separate

DME can occur with NPDR or PDR. Treating central fluid and suppressing peripheral neovascular drive are different decisions; PRP is not a substitute for macular assessment.

A dry OCT can coexist with ischaemia

Central fluid versus lost capillary perfusion

Central subfield shown by inner circle



A dry structural OCT does not exclude ischaemia
FA / OCTA assesses capillary perfusion

Original schematic contrasts where fluid lies with the capillary network around the fovea. It is not an OCT or angiogram from a patient.

When vision remains poor after oedema resolves

- 1 Recheck refraction, lens, retinal structure and optic nerve.
- 2 If macular ischaemia is suspected, assess capillary perfusion with FA or OCT angiography.
- 3 Explain that resolving fluid does not necessarily restore tissue lost to ischaemia.

Good-vision observation is active care

Agree a review schedule using acuity and OCT, and a plan if vision declines. A patient should understand symptom reporting; observation does not mean cancellation of retinal follow-up.